FOR EDITH · PAEDIATRICS & CHILD HEALTH JUNIOR CLERKSHIP

Paediatric History Taking & Communication

Clinical skill | Week 1 · Breathing, fever & the clinical methods | Dr Nabbale, Dr Mayega

The paediatric history is usually taken from a caregiver, covers the whole child (birth, feeding, immunisation, growth, development, HIV exposure and social circumstances) and is the foundation of every diagnosis. Good communication with parents and children, including adolescents seen alone with HEEADSSS, is examined in every OSCE and is how you earn a family's trust on the ward.

Must know before the ward round

1. **Triage before history**: a child with emergency signs (ABCD) is treated first; take a brief focused history while resuscitating, then the full history later.
2. **Record the informant** (who, relationship, reliability) and use the **caregiver's words** for the presenting complaint, with **duration** for each symptom.
3. **Always ask the IMCI general danger signs**: unable to drink or breastfeed, vomits everything, convulsions, lethargic or unconscious.
4. **Paediatric extras** beyond the adult history: **perinatal**, **feeding/nutrition**, **immunisation** (check the card), **growth**, **development**, and **HIV exposure** (mother's status, ART, infant prophylaxis, EID results).
5. **Perinatal history**: ANC attendance, maternal HIV/syphilis/malaria/hypertension, gestation, place and mode of delivery, **birth weight**, **cried at birth** or needed resuscitation, NICU admission, jaundice.
6. **Feeding**: breastfeeding initiated within 1 hour, **exclusive breastfeeding to 6 months**, complementary foods from **6 months**, meal frequency and dietary diversity (24-hour recall), feeding during illness.
7. **Immunisation (UNEPI)**: birth BCG, OPV0, HepB; **6, 10, 14 weeks** OPV, pentavalent, PCV, rotavirus (6, 10 per UCG 2023; 6, 10, 14 on the 2025/26 MoH poster, brand-dependent) and IPV (6, 14); **9 months** MR and yellow fever; **18 months** MR2; malaria vaccine where introduced. Look for the **BCG scar**.
8. **Key milestones**: social smile **6 weeks**, head control **3–4 months**, sits unsupported **6–8 months**, walks **12–15 months**; **refer if not walking at 18 months**, no words at 18 months, or any **loss of skills**.
9. **Adolescents**: see them **alone** for part of the consultation, explain **confidentiality and its limits** (risk of serious harm), and use **HEEADSSS**: Home, Education/employment, Eating, Activities, Drugs, Sexuality, Suicide/mood, Safety.
10. **Communication tools**: open then closed questions, **ICE** (ideas, concerns, expectations), signposting, summarising, avoiding jargon, **teach-back**; break bad news with **SPIKES**.
11. **Child protection red flags**: inconsistent or changing story, delay in seeking care, injury not fitting the child's development, repeated injuries. Act: admit if unsafe, document, involve the senior, social worker/probation officer; Uganda child helpline **116**.
12. **Present the child** with age, sex, weight and nutritional status (Z-scores/MUAC), immunisation status, HIV exposure and feeding, then the problem list.

1 Purpose and Indications
2 Contraindications and Precautions
3 Relevant Anatomy and Normal Values
4 Equipment and Preparation
5 Procedure Step by Step
6 Findings and Interpretation
7 Clinical Acumen
8 Complications and Troubleshooting
9 Documentation and Presenting Findings
10 OSCE Checklist
11 Key Points
12 References

1. Purpose and Indications

In paediatrics the history is taken **with and through a caregiver**: usually the mother, sometimes a father, grandmother, older sibling or guardian, and increasingly the child or adolescent themselves. It is longer than an adult history because a child's illness can only be understood against **how they were born, how they have been fed, whether they are protected by vaccines, how they are growing and developing, whether they have been exposed to HIV and TB, and the family and social world they live in**.

Purpose

- **Make the diagnosis.** The history usually gives you the diagnosis; the examination and tests confirm it.
- **Decide urgency.** The IMCI danger signs and the age of the child decide how fast you act and whether to refer.
- **Protect others.** In Uganda the history also tells you whether siblings are at risk (TB, HIV, measles).
- **Make a plan the family can follow.** It tells you what the family can afford, how far they live from care and whether the child can go home safely.
- **Build trust.** Good communication is how you earn a family's trust on the ward, and it is examined in every OSCE.

> CLINICAL PEARL
> The best paediatric historians **listen first** ("Tell me what has been happening with Brian"), **then structure** (IMCI danger signs, then the system history, then the paediatric extras), and **finish by checking understanding** ("What worries you most about him?"). Parents usually know when something is seriously wrong: take their concern seriously.

> UGANDA CONTEXT
> On Ugandan wards, histories are often taken in **Luganda, Runyankole, Lusoga or another local language**, through an interpreter (sometimes a nurse or a relative). The **Child Health Card** (immunisation, growth chart, vitamin A and deworming record) and any **referral note** from the HC III/IV are part of the history: always ask to see them. Ask about **traditional or herbal treatment** and **"ebiino" (false teeth) extraction** in infants with diarrhoea or fever, which still causes bleeding, sepsis and tetanus in some communities.

Indications

- **Every child seen in clinic or on the ward** needs a history; its depth depends on urgency.
- **Emergency (ETAT)**: a short, focused **SAMPLE** history during resuscitation, and the full history once the child is stable (section 2).
- **Follow-up visits**: an interval history (what has changed, adherence, new symptoms, growth since the last visit).
- **Adolescents (10–19 years)**: a history with the parent plus a confidential **HEEADSSS** psychosocial history with the young person alone (section 5, step 15).
- **Communication tasks that grow out of the history**: explaining a diagnosis, breaking bad news, the angry parent, child protection, discharge counselling and presenting to colleagues (sections 8 and 9).

2. Contraindications and Precautions

There is no contraindication to taking a history, but there are situations where the **full** history must wait or must be taken differently.

- **Emergency signs (ETAT): treat first.** If the child has emergency signs, **treat first**, then take a focused history (**SAMPLE**: **S**ymptoms, **A**llergies, **M**edications, **P**ast illness, **L**ast meal, **E**vents) while resuscitating, and the full history once stable. A colleague can take the SAMPLE history while you treat.
- **Sensitive history**: **do not take a sensitive history** (sexual activity, abuse, HIV disclosure) in front of others without consent, or from a child who is too distressed: arrange a safe, private setting.
- **The caregiver is not the usual carer** (e.g. a neighbour brought the child): record this, take what history you can, and get the full history from the parent later by phone or at the next visit.
- **Interpreters**: use a trained interpreter where possible. **Avoid using a sibling (or another child) for sensitive topics.**
- **Confidentiality**: never ask the mother's HIV status, or an adolescent about sex, in front of other patients or relatives. Ask privately and respectfully.
- **Child protection concerns**: listen and record; **do not confront or accuse** the family yourself (section 8).

3. Relevant Anatomy and Normal Values

Anatomy is not applicable to history taking. This section holds the **normal reference values** that the history is judged against: age bands, duration thresholds, feeding norms, the UNEPI schedule and developmental milestones.

Age bands and duration thresholds

Item	Value	Why it matters
Young infant (IMCI)	**0–2 months**	Different danger signs and fast-breathing cut-off (≥60/min)
Adolescence (WHO)	**10–19 years**	See alone for part of the visit; HEEADSSS
Age of majority (Uganda)	**18 years**	Consent, defilement law
Correct for prematurity	Until **2 years** of age	Subtract the weeks born early before judging milestones and growth
Prolonged cough	**>2 weeks**	Think TB
Persistent diarrhoea	**≥14 days**	HIV, malnutrition
Prolonged fever	**≥7 days**	Typhoid, TB, UTI, abscess
Prolonged rupture of membranes	**>18 hours**	Neonatal sepsis risk
Acute illness	**<2 weeks**	Acute vs persistent pathway

Feeding norms (WHO, UCG 2023)

Item	Norm
Breastfeeding initiation	Within **1 hour** of birth; colostrum given; no pre-lacteal feeds
Exclusive breastfeeding	**0–6 months**
Breastfeeding frequency in young infants	**≥8–12 times in 24 hours**, including night feeds
Complementary feeding	From **6 months** (WHO 2023 guideline on complementary feeding)
Meal frequency	**6–8 months: 2–3 meals**; **9–23 months: 3–4 meals + 1–2 snacks**
Dietary diversity	At least **5 of 8 food groups** in the past day
Animal-source foods	Daily where possible; limit sugary drinks

UNEPI immunisation schedule (2022, plus 2025 malaria vaccine)

Age	Vaccines
Birth	BCG, OPV0, Hepatitis B birth dose
6 weeks	OPV1, DPT-HepB-Hib1, PCV1, Rota1, IPV1
10 weeks	OPV2, DPT-HepB-Hib2, PCV2, Rota2
14 weeks	OPV3, DPT-HepB-Hib3, PCV3, IPV2 (+ Rota3 on the 2025/26 MoH poster, brand-dependent)
6, 7, 8 and 18 months	Malaria vaccine (R21) in districts where it has been introduced Verify *current UNEPI malaria vaccine schedule and districts*
9 months	Measles–rubella 1, yellow fever
18 months	Measles–rubella 2
10 years (girls)	HPV

Also recorded on the card: **vitamin A** (from 6 months, 6-monthly) and **deworming** (from 1 year, 6-monthly).

> UGANDA CONTEXT
>
> **Rotavirus doses**: UCG 2023 lists rotavirus at **6 and 10 weeks**; the **2025/26 MoH immunisation poster** shows a third dose at **14 weeks**, depending on the vaccine brand in use. Check the card and the poster on your ward. Verify *current rotavirus brand and number of doses in UNEPI*

Developmental milestones

Cover **four domains**: **gross motor**, **fine motor and vision**, **speech, language and hearing**, **social, emotional and self-care**. **Correct for prematurity** up to 2 years.

Age	Gross motor	Fine motor / vision	Speech / hearing	Social
6 weeks	Lifts head briefly prone	Follows face/light	Startles to sound	**Social smile**
3–4 months	**Head control**; lifts chest prone	Hands open, reaches; holds rattle	Coos, turns to voice	Laughs
6–8 months	**Sits without support** (6–8 m); rolls	Palmar grasp, transfers objects	Babbles ("baba")	Stranger anxiety (7–9 m)
9–10 months	Crawls, pulls to stand	**Pincer grasp** (9–12 m)	"Mama/dada" non-specific	Waves bye-bye, plays peek-a-boo
12 months	**Walks with/without support** (12–15 m)	Neat pincer, releases objects	**2–3 words** with meaning	Drinks from cup, points
18 months	Walks well, runs, climbs stairs holding on	Tower of 3–4 cubes, scribbles	**6–20 words**; points to body parts	Feeds self with spoon, symbolic play
2 years	Runs, kicks a ball	Tower of 6–7, copies a line	**2-word phrases**, 50+ words	Parallel play, partly toilet trained
3 years	Jumps, stands on one foot briefly, rides tricycle	Copies a circle, tower of 9	Short sentences, knows name, age	Plays with others, dry by day
4 years	Hops, goes down stairs one foot per step	Copies a cross/square, draws a person	Tells stories, asks "why"	Dresses with help, has friends
5 years	Skips	Copies a triangle	Fluent speech	Understands rules, dresses alone

Developmental red flags (refer): no social smile by **8 weeks**; no head control by **4 months**; not sitting by **9 months**; **not walking by 18 months**; no words by **18 months**; no 2-word phrases by **2.5 years**; hand preference before 12–18 months (suggests weakness of the other side); **loss of any previously acquired skill**; parental concern about hearing or vision.

4. Equipment and Preparation

Equipment

Item	Why
A quiet, private space with chairs at the same level	Trust, confidentiality, reduces fear
Toys, picture book or a pen torch to engage the child	A calm child is easier to observe and examine
Child Health Card / Mother–Child Health Passport	Immunisation record, growth chart, birth weight, vitamin A and deworming
Referral note and previous discharge forms	Drugs given, test results, previous diagnoses
Clerking sheet (or structured notebook)	Completeness
WHO growth charts (weight-for-age, length/height-for-age, weight-for-length/height, head circumference) and **MUAC tape**	Plot growth while you take the history
A watch with a second hand (or phone timer)	Count RR while the child is calm on the mother's lap, before you examine
Interpreter (trained where possible)	Accurate history; avoid using a sibling for sensitive topics
Tissues, water	Distressed parents

Preparation, consent and positioning

1. **Read the triage sheet and referral note**; check for **emergency signs** first. If present, treat first (section 2).
2. **Hand hygiene** before you approach.
3. **Setting**: a quiet, private space; sit at the same level as the caregiver; silence your phone.
4. **Consent**: explain who you are, what you will do and how long it will take; ask permission to take notes. For a sensitive history, ask permission before each sensitive area. For adolescents, explain **confidentiality and its limits** at the start (section 5, step 15).
5. **Positioning**: let the child stay **on the parent's lap**; give a toy. Get down to the child's level.
6. **Exposure**: none is needed for the history, but **observe** the child while you talk (step 1 below). Hutchison: the examination begins with the history.
7. **Interpreter**: brief them first (confidentiality, word-for-word translation, no summarising); speak to the caregiver, not to the interpreter.

5. Procedure Step by Step

The order below follows the structure of **Hutchison's Clinical Methods (24th edition)** for the history, with the paediatric additions placed after the past medical history: **presenting complaint → history of presenting complaint → past medical history → birth (pregnancy, delivery, neonatal) → feeding → immunisation → development → drugs and allergies → family history → social history → systems review → closing** Verify *exact sequence of the paediatric history sections in Hutchison 24th edition*. In a timed station, cover every heading briefly rather than one heading in depth.

Step 1. Introduction, informant and observation

1. Wash your hands. **Greet** the caregiver and the child by name; introduce yourself ("I am Edith, a medical student working with Dr …"), and explain what you will do and how long it will take.
2. **Establish who the informant is**: name, relationship to the child, whether they live with the child and how well they know the illness. Ask permission to take notes.
3. Make the child comfortable: let them stay on the parent's lap; give a toy.
4. **Observe while you talk**, before touching: colour (pallor, cyanosis, jaundice), breathing (rate, indrawing, grunting, nasal flaring), alertness and interaction with the mother, posture, wasting or oedema, dysmorphic features, skin rashes, how the child and carer interact (warmth, fear). Count the **respiratory rate** for a full minute while the child is calm.

Step 2. Biodata

Name, **age** (date of birth: in months for infants, exact days for neonates), sex, address (village, parish, district: tells you **malaria transmission**, distance and referral route), religion and tribe (context, not stereotypes), **informant and relationship**, date of admission, inpatient number.

Step 3. Presenting complaint(s)

- In the **caregiver's own words**, with **duration**: "Fever for 3 days, cough for 2 days, convulsions once this morning."
- Order them **chronologically** (or by importance).

Step 4. History of presenting illness (HPI)

1. Start **open**: "Tell me what has been happening since he became unwell."
2. Then **clarify each symptom** (onset, duration, progression, severity, pattern, associated features, relieving/aggravating factors). For pain, adapt **SOCRATES**; in young children ask how they show pain (crying, refusing to walk, pulling the ear).
3. **Always ask the IMCI general danger signs**: **unable to drink or breastfeed? vomits everything? convulsions? lethargic or unconscious?**
4. **IMCI main symptoms**: cough or difficult breathing; diarrhoea (duration, blood, dehydration signs); fever (duration, malaria risk, measles in last 3 months); ear problem; and in young infants, feeding problems, jaundice, umbilical redness, skin pustules.
5. **Systems relevant to the complaint**: e.g. fever → rash, neck stiffness, urinary symptoms, ear discharge, joint swelling.
6. **What has been done so far**: where they went (drug shop, clinic, traditional healer), **what treatment was given** (names, doses, how many days), tests done and results, and the response.
7. **Effect on the child**: feeding, urine output (wet nappies), activity, sleep, school.
8. **Pertinent negatives**: say what is absent (e.g. "no convulsions, no blood in stool").
9. **Contacts**: anyone else sick at home (TB cough, measles, diarrhoea), recent travel.

Step 5. Past medical history

- Previous illnesses and **admissions** (dates, diagnoses, treatment), **surgeries**, **blood transfusions** (number, when: think sickle cell, HIV, haemolysis), chronic illnesses (asthma, epilepsy, sickle cell disease, heart disease, HIV, TB), previous TB treatment.
- **Similar episodes** in the past.

Step 6. Birth (perinatal) history

Essential in children **under 2 years**, and in any child with **developmental or neurological problems**.

Period	Ask	Why it matters
Antenatal	Planned pregnancy? Number of **ANC visits**, maternal **HIV, syphilis (RPR), hepatitis B**, malaria and IPTp, **hypertension/pre-eclampsia**, **diabetes**, fever or rash, drugs, alcohol, smoking, herbal medicine, bleeding, twins	Congenital infections, IUGR, prematurity, HIV exposure, fetal alcohol
Labour and delivery	**Gestation** at birth, **place** (home, HC, hospital) and attendant, **mode** (SVD, assisted, CS and why), duration of labour, prolonged rupture of membranes (>18 h), maternal fever, meconium	Birth asphyxia, neonatal sepsis risk, birth trauma
At birth	**Birth weight**, **cried immediately?** Needed **resuscitation** (bag and mask?), Apgar if known	Low birth weight, hypoxic-ischaemic encephalopathy
Neonatal period	**NICU/SCU admission** and why, **jaundice** (phototherapy, exchange transfusion), convulsions, feeding difficulties, infections, oxygen, length of stay, vitamin K, eye ointment, cord care	Cerebral palsy, hearing loss, kernicterus, chronic lung disease

HIV exposure belongs here and in the feeding history: mother's status and when diagnosed, ART and adherence, last viral load, infant prophylaxis, breastfeeding, and **EID (DNA PCR) results**.

Step 7. Feeding and nutrition history

- **Breastfeeding**: initiated **within 1 hour**? **Colostrum** given? **Pre-lacteal feeds** (water, sugar water, cow's milk, herbs)? **Exclusive breastfeeding** to 6 months? Frequency (≥8–12 times in 24 h in young infants), night feeds, attachment problems, breast problems, maternal HIV status and ART adherence.
- **Replacement feeding**: what milk, how prepared (dilution, clean water, bottles vs cup), how much and how often, cost.
- **Complementary feeding** (WHO 2023 guideline on complementary feeding): started at **6 months**? What foods: porridge (thin or thick?), mashed matooke, beans, groundnut paste, eggs, fish/meat, vegetables and fruit; **meal frequency** (6–8 months: 2–3 meals; 9–23 months: 3–4 meals + 1–2 snacks); **dietary diversity** (at least **5 of 8 food groups** in the past day); **animal-source foods**; sugary drinks.
- **24-hour dietary recall**: "Tell me everything she ate and drank yesterday, from waking up."
- **Feeding during illness**: continued or stopped? Appetite now?
- **Older children**: meals per day, school lunches, food insecurity at home, pica (soil eating: iron deficiency, worms).
- **Growth**: ask about weight loss, clothes becoming loose; **look at the growth chart** on the card.

Step 8. Immunisation history

1. **Check the Child Health Card**; if no card, ask by site and age ("an injection in the right arm at birth that left a scar", "drops in the mouth", "injections in the thighs at 6, 10 and 14 weeks", "an injection in the arm at 9 months").
2. **Look for the BCG scar** (right upper arm).
3. Compare with the **UNEPI schedule** (section 3).
4. Record **vitamin A** (from 6 months, 6-monthly) and **deworming** (from 1 year, 6-monthly).
5. State the conclusion: "**Fully immunised for age**", "**partially immunised** (missed MR at 9 months)", or "**unknown, no card**". Missed vaccines are given at discharge.

Step 9. Developmental history

1. Ask: "**When did she first smile at you? Hold her head up? Sit without support? Walk? Say her first words?**"
2. Ask about regression: "**Is there anything he could do before that he can't do now?**" (regression is always a red flag).

3. Cover **four domains**: **gross motor**, **fine motor and vision**, **speech, language and hearing**, **social, emotional and self-care**. Ask about school performance in older children.
4. **Correct for prematurity** up to 2 years (subtract the weeks born early).
5. Compare with the milestones table and red flags in section 3.

Step 10. Drug and allergy history

Current medicines (including **ART, cotrimoxazole, TB drugs, anticonvulsants**), doses and adherence, recent antibiotics and antimalarials, **herbal and traditional remedies**, and **allergies** (what happened).

Step 11. Family history

- Parents: **ages, occupation, education, health**, **HIV status and treatment**, alive or dead (cause), relationship (living together? polygamous household?), **consanguinity**.
- Siblings: number, ages, health, **deaths and causes** ("Have you lost any children?" Ask gently: a sibling death from neonatal sepsis, sickle cell or HIV changes your thinking).
- **Inherited and familial conditions**: sickle cell disease (or "a sibling who needed frequent transfusions"), epilepsy, asthma/atopy, diabetes, heart disease, bleeding disorders, deafness.
- **Infectious contacts**: **TB in the household** (anyone coughing for >2 weeks or on TB treatment), measles, hepatitis B.
- Draw a **simple family tree (genogram)** where relevant.

Step 12. Social history

- **Who lives at home** and who is the main carer when the mother works; who brought the child today.
- **Housing**: number of rooms and people (crowding: TB, pneumonia), **cooking fuel** (charcoal/firewood indoors: pneumonia, asthma, burns), **water source** and treatment, **toilet** type (diarrhoea, worms), **mosquito net use** last night, smoking at home.
- **Income and occupation**, ability to buy food and medicines, health insurance (rare), **distance and transport** to the facility (affects follow-up and when to return).
- **School**: class, performance, absence, bullying.
- **Child's activities and care**: day care, alone at home, older siblings caring.
- **Pets and animals** (rabies, brucellosis from raw milk).
- **Social support** and stresses (separation, domestic violence, alcohol, mental illness in the carer).

Step 13. Review of systems

A brief run through each system not already covered: general (fever, weight loss, night sweats), skin, eyes, ENT (ear discharge), respiratory, cardiovascular (sweating with feeds, fast breathing, blue spells), gastrointestinal, genitourinary (urine stream, wetting), musculoskeletal, neurological (headache, fits, weakness), endocrine (polyuria, polydipsia).

Step 14. Closing the history

1. **Summarise** back to the caregiver ("So, Brian has had fever for 3 days and a cough for 2, and this morning he had a fit lasting about 5 minutes … have I got that right?").
2. Ask **ICE**: "What do you think is causing this? What worries you most? What were you hoping we would do today?"
3. Ask "**Is there anything else** you want to tell me?"
4. Thank them and explain what happens next (examination, tests, the doctor's review).

Throughout: use **open then closed questions**, **signposting** ("Now I am going to ask about his birth"), **summarising**, plain language without jargon, and empathic responses.

Step 15. The adolescent history: HEEADSSS

Adolescents (10–19 years) should be seen **with the parent** for the main history and **alone** for part of it. Explain **confidentiality and its limits** at the start: "What you tell me stays between us, unless I am worried that you or someone else is at risk of serious harm; if so, I would talk with you first about who needs to know."

HEEADSSS is the expanded form of the original **HEADSS** psychosocial screen; both cover the same ground.

Domain	Example questions
Home	Who do you live with? How do you get on? Have you ever had to leave home? Do you feel safe at home?
Education / employment	Which class? How is school going? Any repeated years, suspensions, bullying? Do you work to earn money?
Eating	How do you feel about your body/weight? Any dieting, vomiting, skipping meals? Enough food at home?
Activities	What do you do for fun? Friends? Sports? Phone and social media use?
Drugs	Do any of your friends drink alcohol, smoke, chew miraa (khat) or use marijuana (bhang)? Do you?
Sexuality	Are you in a relationship? Have you ever had sex? Was it consensual? Do you use condoms or contraception? Any STI symptoms? Pregnancy worries?
Suicide / depression (mood)	How is your mood? Do you ever feel hopeless? Have you ever thought of hurting yourself or ending your life?
Safety	Has anyone ever hurt you or touched you in a way you did not want? Road safety, violence, online safety

Start with the **least threatening** domains (Home, Education) and move to the more personal ones, as recommended in the RCH Melbourne adolescent guideline. For adolescents living with HIV, also ask about **disclosure** (do they know their status?), **adherence**, viral load and stigma.

> **UGANDA CONTEXT**
>
> Uganda's age of majority is **18 years**; the **Children Act (Cap 59, as amended 2016)** protects children from abuse and requires that their best interests guide decisions Verify *chapter number in the 2023 revised edition of the Laws of Uganda*. Ugandan HIV testing policy allows **adolescents to consent to HIV testing** from a younger age than for other procedures (commonly cited as 12 years when they understand the implications) Verify *age in the current MoH HIV Testing Services policy*. Defilement (sex with a person under 18) is a crime: a sexually active young adolescent may need a protection assessment as well as health care. **Sauti 116** is the national child helpline.

6. Findings and Interpretation

What each part of the history tells you

Part of the history	Finding	What it means
Age	Febrile 3-week-old	Sepsis, meningitis
	Wheezy 6-month-old	Bronchiolitis
	Limping 5-year-old	Septic arthritis, sickle cell
	14-year-old with weight loss	TB, HIV, diabetes, eating disorder
Duration	Acute (<2 weeks) vs persistent	Sets the pathway

Part of the history	Finding	What it means
	Cough >2 weeks	TB
	Diarrhoea ≥14 days	Persistent diarrhoea (HIV, malnutrition)
	Fever ≥7 days	Typhoid, TB, UTI, abscess
Danger signs	Any IMCI general danger sign	Decides **urgency and referral**
Treatment already received	Antimalarials, antibiotics, herbs	Changes interpretation (RDT positive after recent antimalarials, partially treated meningitis) and avoids duplicate dosing
HIV exposure	Mother positive or unknown	Changes the differential (PCP in a 3-month-old with severe pneumonia; TB; persistent diarrhoea; failure to thrive) and the tests (DNA PCR)
Perinatal history	Prematurity, asphyxia, jaundice needing exchange	Explains developmental delay, cerebral palsy, deafness and recurrent wheeze in ex-preterms
Feeding history	Early cessation of breastfeeding, diluted milk, thin porridge, low diversity	Explains malnutrition, anaemia and diarrhoea; the most important part of the history in a child with poor growth
Immunisation	Missed or unknown doses	Changes the likelihood of measles, pertussis, diphtheria, tetanus, Hib/pneumococcal meningitis and the decision for LP after a febrile seizure
Development	Delay or regression	Detects disability early and sets expectations
Family history	Sibling deaths, sickle cell, TB contact	Inherited disease, infectious risk to the child and siblings
Social history	Crowding, indoor charcoal, no net, food insecurity, distance	Decides whether a plan is realistic (can the family afford RUTF, return for review, store medicines safely?) and detects neglect and abuse

Patterns to recognise

- **Fever + convulsions + coma** in a malaria area → cerebral malaria, meningitis, hypoglycaemia.
- **Cough + wheeze in infancy** → bronchiolitis or asthma.
- **Persistent diarrhoea + weight loss + oral thrush** → HIV.

Worked examples: turning a history into a differential

- "Fever for 3 weeks, cough, night sweats, weight loss, grandfather coughing for months" → **TB** (with HIV, malnutrition and typhoid as differentials).
- "Wheeze and cough in a 4-month-old, ex-preterm 32 weeks, first episode, coryza, sibling with a cold" → **bronchiolitis**.
- "Sudden screaming episodes, drawing up legs, vomiting, redcurrant jelly stool in a 9-month-old" → **intussusception** (urgent ultrasound and surgical review).

Bedside tests the history prompts

Malaria RDT (fever), **glucose** (convulsions, lethargy, poor feeding), **HemoCue Hb** (pallor, sickle cell), **urine dipstick** (fever without focus, urinary symptoms, oedema), **HIV rapid test** for the mother or child (unknown status), and **plot the growth chart** from the card's previous weights.

Laboratory tests and imaging the history prompts

Test	When (from the history)	Expected / abnormal finding	How it changes management
HIV DNA PCR / rapid test	Mother HIV-positive or unknown status; recurrent infections, failure to thrive, oral thrush	Positive PCR (<18 months) or antibody (≥18 months)	ART, cotrimoxazole; testing of siblings (HC III and above via hub)
Xpert MTB/RIF Ultra (sputum, gastric aspirate, stool), **CXR**	Household TB contact, cough >2 weeks, weight loss	MTB detected; hilar nodes	TB treatment or TPT for contacts (HC IV and above for CXR)
Hb electrophoresis / POC sickle test	Family history of sickle cell, recurrent painful swellings, transfusions	HbSS	Sickle cell clinic, penicillin V, folic acid, hydroxyurea (regional/national referral)
Hearing assessment	Neonatal jaundice needing exchange, meningitis, speech delay	Hearing loss	Audiology, hearing aids, speech therapy (national referral)
Developmental assessment (formal tools, e.g. Malawi Developmental Assessment Tool, used in some Ugandan units)	Delayed milestones on history	Domain-specific delay	Physiotherapy, occupational therapy, special education (regional/national)
Thyroid function	Prolonged neonatal jaundice, constipation, delay, large tongue	Low T4, high TSH	Levothyroxine (national referral)
Stool microscopy	Chronic diarrhoea, pica, worms seen	Ova, cysts	Antihelminthics

Link the history to a **targeted examination**: weight, length/height, MUAC, oedema and Z-scores (see General Examination & Growth Assessment), vital signs against age norms, then the system suggested by the history. Confirm historical findings ("he looks yellow") with your own examination.

7. Clinical Acumen

The order of approach: making the history serve the child's safety

First 5 minutes

1. **Look before you talk**: triage the child (ETAT). Emergency signs → call for help and treat (airway, oxygen, glucose, fluids, anticonvulsant) while a colleague takes a **SAMPLE** history.
2. If no emergency signs, check **priority signs** (tiny baby <2 months, temperature very high, trauma, pallor, poisoning, pain, respiratory distress, restless/irritable/lethargic, referral, malnutrition, oedema of both feet, burns): these children are assessed next, ahead of the queue.
3. Settle the child on the caregiver's lap; count the respiratory rate before the child is disturbed.

The focused history checklist (each item changes something):

- [ ] Informant and their reliability.
- [ ] Presenting complaints with durations.
- [ ] **Danger signs** (changes urgency).
- [ ] **Treatment already given** (changes drugs and tests).
- [ ] **HIV exposure and TB contact** (changes the differential and testing).
- [ ] **Feeding and growth** (changes nutritional management).

- [ ] **Immunisation** (changes differential and discharge plan).
- [ ] **Development** (changes long-term plan).
- [ ] **Social situation** (changes discharge and follow-up).

Red flags in the history: when to stop and escalate

RED FLAG

- **IMCI danger signs**: unable to drink/breastfeed, vomits everything, convulsions, lethargy/unconsciousness.
- **Young infant (<2 months)**: not feeding well, convulsions, fast breathing (≥60/min), severe chest indrawing, fever ≥37.5°C or temperature <35.5°C, movement only when stimulated.
- **Regression** of developmental skills.
- **Bilious (green) vomiting** (malrotation, obstruction), **blood in stool with irritability** (intussusception).
- **Weight loss, night sweats, cough >2 weeks** with a TB contact.
- **Child protection concerns**: history that changes or does not explain the injury, delay in seeking care, injury inconsistent with development ("he rolled off the bed" in a 2-week-old with a fracture), repeated injuries, a frightened or withdrawn child, disclosure by the child.
- **Caregiver distress or mental illness** that affects the child's safety.

Thinking like a clinician

- **The history comes from someone else**: ask how they know ("Did you see the fit yourself?"), and distinguish observation from interpretation ("he had malaria" may mean "he had fever").
- **Translate local terms carefully**: "*omusujja*" (fever, often equated with malaria), "*ensimbu*" (convulsions), "*ekifuba*" (chest problem/cough), "*ebiino*" (false teeth). Ask what the caregiver means.
- **If X, then think Y**: fever + convulsions + coma in a malaria area → cerebral malaria, meningitis, hypoglycaemia; cough + wheeze in infancy → bronchiolitis or asthma; persistent diarrhoea + weight loss + oral thrush → HIV.
- **"If the history is unclear, ask a different person or ask again later."** A grandmother may know the birth history, the father may know about the TB contact.
- **Parents usually know** when something is seriously wrong: take their concern seriously.
- **Judgement call: how long a history?** A child with emergency signs gets SAMPLE now and the full history later; a well child in clinic gets the full history; a timed OSCE gets every heading briefly.

Common student mistakes

Starting with closed questions; forgetting the danger signs; not asking for the card; accepting "fully immunised" without checking; skipping the feeding history in a malnourished child; not asking the mother's HIV status (or asking it publicly and insensitively); forgetting to ask what treatment has been given; asking an adolescent about sex in front of parents; presenting a child without the weight.

8. Complications and Troubleshooting

What goes wrong when the history is poorly taken or communicated

- **Missed diagnoses**: TB (no contact history), HIV (no maternal status), intussusception (no stool description), developmental disorders (no milestones).
- **Medication errors**: duplicate antimalarials, wrong dose because the weight or previous treatment was not recorded, missed allergy.

- **Missed child protection** or neglect.
- **Loss of trust**: families who feel judged or unheard may leave against advice or not return.
- **Breaches of confidentiality**: an adolescent or mother whose HIV status is disclosed without consent may suffer stigma, violence or abandonment.
- **Poor adherence** after discharge when instructions were not understood.

Troubleshooting

Problem	What to do
Informant did not witness the events	Record it; ask who did; phone the parent or ask at the next visit
History unclear or contradictory	Ask a different person or ask again later; distinguish observation from interpretation
No Child Health Card	Ask by age and site, look for the BCG scar, record "unknown (no card)", follow the catch-up schedule
Language barrier	Trained interpreter; brief them; speak to the caregiver; avoid a sibling for sensitive topics
Child too distressed or a sensitive topic	Move to a private, safe setting; come back later
Child has emergency signs	Stop; treat; SAMPLE history; full history later

Communication tasks that follow the history

In a history and communication skill, "management" means handling the conversations and situations that arise from the history, and turning the history into a safe plan the family understands.

1. Explaining a diagnosis and plan (the ward-round conversation)

1. **Check what they know**: "What have the doctors told you so far?"
2. **Give information in small chunks**, in plain language, in the family's language, avoiding jargon ("infection of the lungs" rather than "lower respiratory tract infection").
3. **Explain the plan**: what treatment, how long, what the family can do (breastfeeding, giving ORS, feeding).
4. **Check understanding with teach-back**: "To make sure I explained well, can you tell me how you will give the medicine at home?"
5. **Invite questions** and address concerns.
6. **Safety-net**: danger signs that mean they must return immediately, and when to come back for review.

2. Breaking bad news (SPIKES)

1. **S**etting: private room, sit down, phone silenced, both parents or the person they choose present, a nurse or counsellor with you, enough time.
2. **P**erception: "What do you understand about Grace's illness so far?"
3. **I**nvitation: "Would you like me to explain the test results in detail?"
4. **K**nowledge: warning shot ("I am afraid I have some difficult news"), then clear, simple information; avoid euphemisms (say "cancer" or "HIV" if that is the diagnosis); pause.
5. **E**motions: acknowledge and respond with empathy ("I can see this is a terrible shock"); allow silence and tears.
6. **S**trategy and summary: what happens next, what support is available (counsellors, palliative care, social worker), and when you will talk again.

Common situations: a new HIV diagnosis in a child (and therefore usually the mother), a diagnosis of cerebral palsy or leukaemia, a severe disability after meningitis, and the death of a child.

3. The angry or distressed parent

- Stay calm; sit down; **listen without interrupting**; acknowledge the feeling ("You are upset because he waited 3 hours to be seen; I'm sorry about that").
- Find out the real concern (fear for the child, cost, a perceived mistake).
- Explain honestly what happened and what will happen now; **do not blame colleagues**.
- Involve a senior early; if there has been an error, follow **open disclosure** (say sorry, explain, put it right, report it).

4. Adolescent consultations

- See the adolescent **alone** for part of the visit (explain this to parents as routine).
- Explain **confidentiality and its limits** at the start.
- Use **HEEADSSS**; non-judgemental language; respect privacy during examination (chaperone).
- Encourage them to involve a trusted adult; respect their developing autonomy in decisions; for HIV, support **disclosure** (most programmes aim for partial disclosure from about 6–10 years and full disclosure by about 10–12 years, adapted to the child's maturity).

5. Child protection: when the history suggests abuse or neglect

1. **Listen and record** the history verbatim, including who said what, and **dates and times**.
2. **Examine fully** (with a chaperone) and document injuries with a body map; measure growth (neglect).
3. **Ensure the child's immediate safety**: admit if going home would be unsafe.
4. **Inform the senior/consultant** the same day; involve the **social worker**, **probation and social welfare officer**, and police when required (defilement, serious injury). **Sauti 116** child helpline.
5. Do not confront or accuse the family yourself; be honest that you must share concerns to keep the child safe.
6. Treat injuries and infections; consider **post-exposure prophylaxis** (HIV PEP within 72 h, emergency contraception, STI prophylaxis) after sexual assault.

6. Discharge counselling

Diagnosis in simple words; medicines (name, dose in mL or tablets, how often, for how long) with a **demonstration**; feeding plan; danger signs; **follow-up appointment** (date and place); **pending results** (e.g. EID PCR) and how they will be told; missed vaccines given; contact number.

9. Documentation and Presenting Findings

Structure

Use a clear structure: **identification** (name, age, sex, weight, nutritional status, immunisation, HIV exposure, feeding), **presenting complaints**, **key positives and negatives**, relevant **background** (perinatal, development, family, social), **examination findings**, **summary and problem list**, **differentials**, **plan**. For handovers, **SBAR** (Situation, Background, Assessment, Recommendation) is efficient.

Model one-line summary and problem list

"**Baby Nakato**, a **7-month-old girl**, **6.2 kg** (weight-for-length Z-score −2.5, MUAC 11.8 cm, no oedema), **born at term weighing 3.0 kg**, **HIV-exposed** (mother on TLD for 4 years, last VL suppressed; NVP prophylaxis completed; 6-week PCR negative), **partially immunised** (missed 14-week vaccines), **stopped breastfeeding at 4 months** and now fed **diluted cow's milk and thin porridge 2 times a day**, presenting with **watery diarrhoea for 16 days** and poor weight gain."

Problem list: (1) persistent diarrhoea, (2) moderate acute malnutrition with poor complementary feeding, (3) HIV-exposed infant needing repeat DNA PCR, (4) missed vaccines, (5) family food insecurity (father unemployed).

Model case presentation

"**Kato** is a **2-year-and-3-month-old boy** from **Kawempe**, brought by his **mother, a reliable informant**. He weighs **9.8 kg** (weight-for-height Z-score **−2.2**, height-for-age −2.6), **MUAC 12.3 cm**, no oedema. He is **fully immunised for age** by card with a BCG scar, received vitamin A and albendazole 3 months ago, is **HIV-unexposed** (mother tested negative in ANC and 2 months ago), was **breastfed for 14 months** and now eats family foods twice a day (mostly porridge and matooke, rarely beans or animal foods). He presents with **cough for 4 weeks, fever for 2 weeks and weight loss**. His **grandmother, who shares his sleeping room, has had a cough for 3 months** and has not been tested. He has no danger signs. Born at term at Kawempe hospital by SVD, birth weight 3.2 kg, cried immediately. Milestones normal (walked at 13 months, 2-word phrases now). **Summary**: a toddler with chronic cough, fever and moderate wasting with a probable TB contact. **Differentials**: pulmonary TB, HIV-related lung disease (re-test), persistent pneumonia, foreign body. **Plan**: CXR, Xpert Ultra on gastric aspirate or stool, HIV test, nutritional assessment and counselling, contact tracing for the grandmother."

10. OSCE Checklist

Step	Done well when	Critical?
Hand hygiene; greets caregiver and child by name; introduces self and role	Speaks to the child too and gets down to their level	No
Confirms informant and obtains consent	Name, relationship and reliability recorded; permission to take notes	Yes
Checks for emergency/danger signs or acknowledges triage	States that a child with emergency signs is treated first (SAMPLE history)	Yes
Biodata	Age in months for infants (days for neonates); address/district	No
Presenting complaints	In the caregiver's words, each with a duration	Yes
HPI	Open question first, then clarifies each symptom; asks all **4 IMCI danger signs**	Yes
Treatment already given	Drugs, doses, days, traditional remedies, and the response	Yes
Pertinent negatives and contacts	TB, measles, diarrhoea contacts named	No
Past medical history	Admissions, transfusions, chronic illness, surgery	No
Perinatal history	ANC, maternal HIV/syphilis, gestation, mode/place of delivery, birth weight, cried at birth, NICU, jaundice	Yes (under 2 y)
Feeding history	Breastfeeding initiation and exclusivity, complementary feeding, 24-hour recall	Yes
Immunisation history	Asks for the card, states the status, mentions the BCG scar	Yes
Developmental history	Four domains, corrected for prematurity, asks about regression	Yes
Drug history and allergies	Includes ART, cotrimoxazole, herbal remedies; what the allergy did	No
Family history	Parents' health and HIV status, siblings alive/dead, inherited illness, TB contact	No

Step	Done well when	Critical?
Social history	Household, housing, water/sanitation, fuel, net use, income, school, distance	No
Brief review of systems	Covers systems not yet asked	No
Communication skills	Open and closed questions appropriately; signposting; no jargon; empathic responses	Yes
Explores ICE; summarises; checks for anything else	All three done before closing	No
Thanks the family and explains next steps; presents a concise summary with problem list	Opening line includes weight, nutritional, immunisation and HIV status	No

ll. Key Points

High-yield points

- Triage before history: emergency signs are treated first, with a SAMPLE history.
- Record the informant and use the caregiver's words, with a duration for each complaint.
- Always ask the four IMCI general danger signs.
- The paediatric extras: perinatal, feeding, immunisation (check the card), growth, development and HIV exposure.
- Never accept "fully immunised" without the card and the BCG scar.
- Regression of any skill is always a red flag.
- See adolescents alone for part of the visit; explain confidentiality and its limits; use HEEADSSS.
- ICE, signposting, summarising, teach-back; SPIKES for bad news.
- Child protection: listen, document, ensure safety, involve the senior, social worker and probation officer; do not confront the family; Sauti 116.
- Open every presentation with age, sex, weight, nutritional status, immunisation, HIV exposure and feeding.

Numbers & doses

Item	Value
Social smile	**6 weeks** (red flag: none by 8 weeks)
Head control	**3–4 months** (red flag: none by 4 months)
Sits unsupported	**6–8 months** (red flag: not by 9 months)
Pincer grasp	**9–12 months**
Walks	**12–15 months** (red flag: not walking by 18 months)
First words	**12 months**; 2-word phrases **2 years**
Correct for prematurity	Until **2 years** of age
Exclusive breastfeeding	**0–6 months**; initiation within **1 hour** of birth
Complementary feeding	From **6 months**; minimum **5 of 8 food groups** daily; 2–3 meals at 6–8 months, 3–4 + snacks at 9–23 months
Immunisation contacts	Birth, **6, 10, 14 weeks**, **9 months**, **18 months** (+ malaria vaccine schedule where introduced)
Prolonged cough (TB)	**>2 weeks**
Persistent diarrhoea	**≥14 days**

Item	Value
Prolonged rupture of membranes	**>18 hours** (neonatal sepsis risk)
Adolescence (WHO)	**10–19 years**; Uganda age of majority **18**
HIV PEP after sexual assault	Within **72 hours**
Child helpline (Uganda)	**116** (Sauti)

Exam traps

- **Trap:** taking a long history while a child with emergency signs waits. **Right:** triage and treat first; SAMPLE history during resuscitation.
- **Trap:** "Fully immunised" because the mother says so. **Right:** check the card and BCG scar, and list any missed doses.
- **Trap:** asking a 15-year-old about sexual activity in front of her mother. **Right:** see her alone after explaining confidentiality.
- **Trap:** skipping the perinatal and developmental history in a child with seizures. **Right:** birth asphyxia, neonatal jaundice and developmental delay change the diagnosis and prognosis.
- **Trap:** not asking about the mother's HIV status because "it is sensitive". **Right:** ask privately and respectfully; HIV exposure changes management.
- **Trap:** presenting the child without weight, nutritional status and immunisation status. **Right:** these belong in the opening line.
- **Trap:** confronting parents about suspected abuse yourself. **Right:** ensure safety, document, involve senior, social worker and probation officer.

Questions seniors ask

Viva questions

1. *What are the IMCI general danger signs?* Unable to drink or breastfeed, vomits everything, has had convulsions (or convulsing now), lethargic or unconscious.
2. *What is in a perinatal history?* Antenatal (ANC, maternal illnesses including HIV/syphilis, drugs), labour and delivery (gestation, place, mode, complications), birth (weight, cried at birth, resuscitation), neonatal period (NICU, jaundice, infections, feeding).
3. *How do you assess immunisation status without a card?* Ask by age and site, look for the BCG scar, record as "unknown (no card)" and follow the catch-up schedule; never assume "fully immunised".
4. *What are the developmental red flags?* No smile by 8 weeks, no head control by 4 months, not sitting by 9 months, not walking by 18 months, no words by 18 months, early hand preference, any regression, concern about vision or hearing.
5. *What is HEEADSSS and why see adolescents alone?* A psychosocial screen (Home, Education/employment, Eating, Activities, Drugs, Sexuality, Suicide/mood, Safety); adolescents disclose more when confidentiality is assured.
6. *What are the components of SPIKES?* Setting, Perception, Invitation, Knowledge, Emotions/empathy, Strategy and summary.
7. *What features in the history suggest non-accidental injury?* Delay in presentation, inconsistent or changing history, mechanism not matching the injury or development, repeated injuries, a frightened child, disclosure.
8. *Why ask about the cooking fuel?* Indoor smoke from charcoal and firewood is a major risk factor for pneumonia and asthma (and burns).

On the ward round

1. **"Who gave you this history and how reliable is it?"** Name the informant, relationship and whether they witnessed the events.
2. **"Is this child immunised?"** Give the status by card, the BCG scar, and any missed doses and when they will be given.
3. **"What is this child's HIV exposure status?"** Mother's status and ART, viral load, infant prophylaxis, breastfeeding, PCR results.
4. **"How is she fed?"** Breastfeeding history, complementary foods, meal frequency and diversity, 24-hour recall.
5. **"Are her milestones normal?"** Give key milestones in each domain, corrected for prematurity, and any regression.
6. **"What did they already give at home or at the clinic?"** Drugs, doses, timing, herbal treatment.
7. **"Who is at home and can they cope with this plan?"** Household, carers, money, distance, and safety.
8. **"How would you break the news of a positive HIV PCR to this mother?"** SPIKES, privacy, counsellor present, start ART the same day, plan disclosure and partner testing, and arrange support.

Mnemonics

- **Paediatric history order: "BIG PAIN FIDS"**: **B**iodata, **I**nformant, **G**rievances (complaints), **P**resenting illness, **A**ntenatal/perinatal, **I**mmunisation, **N**utrition/feeding, **F**amily, **I**llnesses (past), **D**evelopment, **S**ocial (and drugs). (A memory aid for the content; present in the order of section 5.)
- **Danger signs: "Can't Drink, Vomits All, Fits, Floppy"**.
- **Emergency history: SAMPLE** (Symptoms, Allergies, Medications, Past illness, Last meal, Events).
- **Adolescent: HEEADSSS**.
- **Bad news: SPIKES**.
- **Patient perspective: ICE** (Ideas, Concerns, Expectations).
- **Milestones "6-weeks smile, 6-months sit, 1-year walk, 2-years phrases"** (with ranges and red flags at 8 weeks, 9 months, 18 months, 2.5 years).
- **Handover: SBAR**.

OSCE tips

- **Introduce yourself to the child too** and get down to their level.
- In a timed station, cover **danger signs, HPI, perinatal, feeding, immunisation, development, family/social** briefly; examiners look for the paediatric extras.
- **State the immunisation and nutritional status** in your summary.
- **Use ICE and summarise**: these communication marks are easy to earn and easy to forget.
- With a simulated adolescent, ask the parent to step out before sensitive questions.

12. References

1. Ministry of Health Uganda. Uganda Clinical Guidelines 2023: National Guidelines for Management of Common Health Conditions (IMCI sick child, routine childhood vaccination, nutrition). Kampala: MoH; 2023. https://library.health.go.ug/sites/default/files/resources/Uganda%20Clinical%20Guidelines%202023.pdf
2. World Health Organization. Pocket Book of Hospital Care for Children, 2nd edition. Geneva: WHO; 2013. https://iris.who.int/handle/10665/81170
3. World Health Organization. Updated guideline: paediatric emergency triage, assessment and treatment (ETAT). Geneva: WHO; 2016. https://iris.who.int/handle/10665/204463
4. The Royal Children's Hospital Melbourne. Clinical Practice Guideline: Engaging with and assessing the adolescent patient (HEEADSSS). https://www.rch.org.au/clinicalguide/guideline_index/Engaging_with_and_assessing_the_adolescent_patient/

5. World Health Organization. WHO guideline for complementary feeding of infants and young children 6–23 months of age. Geneva: WHO; 2023.
6. Ministry of Health Uganda. Consolidated guidelines for the prevention and treatment of HIV and AIDS in Uganda. Kampala: MoH; 2022.
7. Lissauer T, Carroll W, editors. Illustrated Textbook of Paediatrics (history taking and development chapters). Elsevier; 2021.
8. Glynn M, Drake WM, editors. Hutchison's Clinical Methods: An Integrated Approach to Clinical Practice. 24th edition. Edinburgh: Elsevier; 2018.

SELF-TEST

Flashcards

31 cards. Cover the right-hand column and test yourself.

#	Question	Answer
1	What do you do before taking a history from a sick child?	**Triage (ETAT)**: emergency signs → treat first, take a **SAMPLE** history during resuscitation; priority signs → assess next.
2	The four IMCI general danger signs	**Unable to drink or breastfeed; vomits everything; convulsions; lethargic or unconscious.**
3	Danger signs in a young infant (<2 months)	Not feeding well, convulsions, fast breathing **≥60/min**, severe chest indrawing, fever **≥37.5°C** or temperature **<35.5°C**, movement only when stimulated.
4	Why record the informant?	The history comes second-hand: note **who**, **relationship**, whether they **witnessed** events and their **reliability**.
5	Antenatal items in a paediatric history	ANC visits, maternal **HIV, syphilis, hepatitis B**, malaria/IPTp, hypertension, diabetes, fever/rash, drugs, alcohol, herbs, bleeding, multiple pregnancy.
6	Birth and neonatal items	Gestation, place and mode of delivery, PROM >18 h, maternal fever, **birth weight**, **cried at birth**/resuscitation, NICU admission, jaundice/phototherapy, convulsions, feeding problems.
7	Breastfeeding questions	Initiation **within 1 hour**, colostrum, **pre-lacteal feeds**, **exclusive to 6 months**, frequency and night feeds, problems, mother's HIV status/ART.
8	Complementary feeding standards (WHO)	Start at **6 months**; 2–3 meals (6–8 m), 3–4 meals + 1–2 snacks (9–23 m); **≥5 of 8 food groups** daily; include animal-source foods; avoid sugary drinks.
9	What is a 24-hour dietary recall?	Ask the caregiver to list **everything the child ate and drank yesterday** from waking to sleeping: quantity, frequency and diversity.
10	UNEPI vaccines at 6, 10 and 14 weeks	**OPV, DPT-HepB-Hib (pentavalent), PCV** at all three; **rotavirus** at 6 and 10 weeks (UCG 2023; the 2025/26 MoH poster adds 14 weeks, brand-dependent); **IPV** at 6 and 14 weeks.
11	UNEPI vaccines at birth, 9 months and 18 months	Birth: **BCG, OPV0, HepB birth dose**. 9 months: **measles–rubella 1 + yellow fever**. 18 months: **MR2**.
12	How do you check immunisation without a card?	Ask by **age and site**, look for the **BCG scar** (right upper arm), record 'unknown (no card)' and plan catch-up. Never assume.
13	Social smile and head control	Social smile **6 weeks** (red flag none by 8 weeks); head control **3–4 months** (red flag none by 4 months).
14	Sitting and walking milestones	Sits unsupported **6–8 months** (red flag not by 9 months); walks **12–15 months** (red flag **not by 18 months**).
15	Speech milestones	Babbles 6 months; **2–3 words at 12 months**; 6–20 words at 18 months; **2-word phrases at 2 years** (red flag no words at 18 months, no phrases at 2.5 years).
16	The most worrying developmental history finding	**Regression** (loss of previously acquired skills): always refer.
17	Early hand preference	Hand preference before **12–18 months** suggests **weakness of the other side** (e.g. hemiplegic cerebral palsy).

#	Question	Answer
18	Correcting for prematurity	Subtract the weeks born early from the age when assessing milestones, **until 2 years**.
19	Family history essentials	Parents' age, health and **HIV status**, consanguinity, **siblings alive/dead and causes**, inherited disease (sickle cell, epilepsy, asthma), **TB in the household**.
20	Social history essentials (Uganda)	Who lives at home/main carer, rooms and crowding, **cooking fuel**, water and toilet, **mosquito net use**, income, school, **distance to facility**, stresses and safety.
21	HEEADSSS	**H**ome, **E**ducation/employment, **E**ating, **A**ctivities, **D**rugs, **S**exuality, **S**uicide/mood, **S**afety. Start with least threatening; see the adolescent alone.
22	How do you explain confidentiality to an adolescent?	'What you tell me stays between us **unless** I think you or someone else is at risk of **serious harm**; then I'd talk with you first about who needs to know.'
23	ICE	Explore the caregiver's **Ideas** (what they think it is), **Concerns** (what worries them), **Expectations** (what they hope we will do).
24	SPIKES	**S**etting, **P**erception, **I**nvitation, **K**nowledge (warning shot, plain words), **E**motions (empathy), **S**trategy and summary.
25	Teach-back	Ask the caregiver to **explain back** in their own words how they will give the medicine or what danger signs to watch for; correct misunderstandings.
26	History red flags for child abuse	Delay in seeking care; **inconsistent or changing story**; mechanism **not fitting the injury or development**; repeated injuries; frightened child; disclosure.
27	What to do if you suspect abuse	Ensure **safety** (admit), document verbatim and body map, inform the **senior**, involve **social worker/probation officer**/police as needed; Uganda helpline **116**; do not confront the family yourself.
28	What is 'ebiino' and why ask?	Traditional extraction of infant canine tooth buds ('false teeth') for diarrhoea/fever in parts of Uganda: causes **bleeding, sepsis, tetanus, anaemia**. Ask about traditional treatments.
29	The opening line of a paediatric case presentation	Name, **age, sex**, **weight and nutritional status** (Z-scores/MUAC), **immunisation status**, **HIV exposure**, feeding, then presenting complaints with durations.
30	SBAR handover	**S**ituation, **B**ackground, **A**ssessment, **R**ecommendation.
31	SAMPLE history in an emergency	**S**ymptoms, **A**llergies, **M**edications, **P**ast illnesses, **L**ast meal, **E**vents leading up.

SELF-TEST

Single best answer questions

16 questions. The answers and explanations follow at the end.

1. A 14-month-old girl is carried into the paediatric emergency room at Mengo by her mother. She is convulsing, and the mother is distressed and wants to explain the 3-day illness. What should the student do FIRST?

A Call for help and start emergency treatment (airway, oxygen, glucose, anticonvulsant) while a brief SAMPLE history is taken

B Take a full history including perinatal and immunisation history

C Ask the mother to wait at triage while you find the child's card

D Take the history from the mother in a private room first

E Send the child for a CT scan

2. A 9-month-old boy is brought by his aunt, who has cared for him since yesterday because his mother travelled. She says 'he has had malaria for 4 days'. What is the MOST important next step in the history?

A Accept the diagnosis of malaria and move on to the past medical history

B Ask only closed questions to save time

C Refuse to take a history until the mother returns

D Ask what she means by 'malaria', what she has seen herself, and record her as the informant with limited knowledge; contact the mother later

E Ask the aunt about the mother's HIV status in the open ward

3. A 4-month-old boy is admitted with fast breathing (RR 64/min), chest indrawing and SpO_2 86%. His mother is quiet and has not mentioned her own health. Which history question MOST changes your immediate management?

A How many teeth does he have?

B What is your HIV status, and are you on treatment?

C Does the family keep animals?

D Which church do you attend?

E When did he first roll over?

4. A 3-year-old girl presents with cough for 5 weeks and weight loss. Her mother says she is 'fully immunised'. There is no card. Which is the BEST way to document immunisation status?

A Write 'fully immunised' as the mother says

B Ask the mother to bring the card next year

C Record 'unimmunised' and give all vaccines again today

D Skip immunisation history because she is over 2 years

E Ask about each vaccine by age and site, look for a BCG scar, and record 'immunisation status unknown, no card; BCG scar present' with a catch-up plan

5. A 10-month-old girl (6.0 kg, weight-for-length Z-score −2.8, no oedema) is brought for poor weight gain. Which part of the history is MOST important for planning her management?

A The father's occupation only

B Family history of asthma

C The age at which she smiled

D A detailed feeding history with 24-hour dietary recall, breastfeeding history and feeding during illness

E The number of rooms in the house only

6. A 15-year-old girl attends with her mother because of abdominal pain and missed periods. Which approach is MOST appropriate for the sensitive part of the history?

A Ask about sexual activity directly in front of her mother

B Explain to both that it is routine to see adolescents alone for part of the visit, then with the girl alone explain confidentiality and its limits and use HEEADSSS

C Avoid asking about sexual activity because she is a minor

D Ask the mother whether her daughter is sexually active

E Order a pregnancy test without telling the girl

7. A 2-year-old boy is not yet walking independently. He was born at 30 weeks and spent 6 weeks in the neonatal unit with oxygen and jaundice needing phototherapy. Which interpretation is MOST appropriate?

Advanced

A He is normal; corrected age is under 15 months, so no concern

B He should be started on anticonvulsants

C Prematurity explains everything; review in 2 years

D Correction for prematurity stops at 12 months

E His corrected age is about 22 months; not walking by 18 months (corrected) is a red flag, so he needs a full developmental and neurological assessment

8. A 4-year-old boy is brought with a spiral fracture of the femur. The father says he fell from a chair yesterday evening; the mother says he was injured while playing outside 3 days ago. He has bruises of different colours on his back. What is the MOST appropriate action?

A Discharge with a plaster and advise more supervision

B Confront the parents and accuse them of abuse

C Treat the fracture, admit for safety, document the histories verbatim with a body map, inform the senior and involve the social worker/probation officer

D Call the police yourself before telling anyone else

E Ignore the bruises because children bruise easily

9. While explaining that her 3-year-old needs admission for severe pneumonia, a mother shouts that she waited 4 hours and the doctors 'do not care'. What is the BEST first response?

A Tell her the hospital is busy and she must be patient

B Sit down, listen without interrupting, acknowledge her frustration and apologise for the wait, then explain the plan

C Leave and ask a nurse to deal with her

D Tell her she is free to go to another hospital

E Blame the triage nurse

10. A mother is told her 8-week-old son's DNA PCR is positive. She becomes silent and starts crying. Using SPIKES, what should you do NEXT?

A Allow silence, acknowledge her emotion with empathy, and give her time before discussing next steps

B Immediately explain the ART regimen and dosing

C Tell her not to cry because HIV is a manageable disease

D Ask her how she got HIV

E End the consultation and ask her to come back next month

11. A 6-month-old baby has had diarrhoea for 3 days. On history the grandmother reports that 'false teeth' were removed by a traditional healer 2 days ago, and the baby now has bleeding gums and fever. What is the MOST important implication of this history?

A None; it is a harmless cultural practice
B The baby needs dental referral only
C The diarrhoea is caused by teething
D Risk of bleeding, sepsis, tetanus and anaemia from 'ebiino' extraction: examine the gums, check Hb, assess tetanus risk and treat infection
E Report the grandmother to the police immediately

12. A 5-week-old girl is seen with jaundice. Which perinatal or neonatal history item MOST changes your next investigation?

A Mother's religion
B Father's occupation
C Birth order
D Whether she has started smiling
E Pale (white or clay-coloured) stools and dark urine

13. A 13-year-old boy living with HIV is admitted with diarrhoea. His viral load is 45,000 copies/mL. During the history alone with you he says he 'takes the medicine sometimes' and doesn't know why he takes it. What is the MOST appropriate response? Advanced

A Tell him he has HIV immediately without involving his caregiver
B Scold him for poor adherence
C Explore barriers to adherence non-judgementally, and work with the caregiver and counsellor towards full disclosure of his HIV status
D Ignore it because adherence is the caregiver's responsibility
E Switch him to second-line ART today

14. On the ward round you present: 'This is Musa, a 2-year-old with fever and cough.' The consultant asks what is missing from your opening line. Which answer is BEST?

A The weight and nutritional status (Z-scores/MUAC), immunisation status, HIV exposure and feeding
B The tribe and religion
C The father's name
D The ward bed number only
E The last vaccine batch number

15. A 7-year-old boy has recurrent cough and wheeze, worse at night. Which social history question is MOST relevant to his management?

A What fuel does the family use for cooking, and where is it used?
B Which school sports does he play?
C Does the family have a radio?
D What is the father's tribe?
E How far is the nearest church?

16. You have explained to a mother how to give amoxicillin dispersible tablets twice daily for 5 days. What is the BEST way to check that she has understood?

- A Ask 'Do you understand?'
- B Give her a printed leaflet in English
- C Ask her to show you how she will prepare a dose and tell you when and for how many days she will give it
- D Tell her the pharmacist will explain
- E Ask her to sign the chart

Answers & explanations

1. A · Call for help and start emergency treatment (airway, oxygen, glucose, anticonvulsant) while a brief SAMPLE history is taken

A is correct: a convulsing child has an **emergency sign**: treat first (ETAT), with a brief **SAMPLE** history (symptoms, allergies, medicines, past illness, last meal, events) during resuscitation; the full history comes once stable. **B** and **D** are wrong: delaying treatment for a full history is dangerous. **C** is wrong: the child needs immediate care, not a queue. **E** is wrong: imaging is not part of emergency management of a convulsing child.

2. D · Ask what she means by 'malaria', what she has seen herself, and record her as the informant with limited knowledge; contact the mother later

D is correct: the informant's **relationship and reliability** must be recorded, and local terms like 'malaria' (*omusujja*) often just mean fever; clarify what was **observed** and fill gaps from the mother later. **A** is wrong: accepting a lay diagnosis risks missing pneumonia, meningitis or UTI. **C** is wrong: the child needs assessment now. **B** is wrong: start with open questions and then clarify. **E** is wrong: HIV status should be asked privately and the aunt may not know or have the right to disclose it.

3. B · What is your HIV status, and are you on treatment?

B is correct: in a young infant with severe pneumonia and hypoxaemia, **HIV exposure** raises the likelihood of **Pneumocystis pneumonia** (needs high-dose cotrimoxazole and prednisolone), changes testing (DNA PCR) and prophylaxis. Ask privately and sensitively. **A** is wrong: teething is irrelevant. **C** is wrong: animal exposure matters less here. **D** is wrong: religion is context, not management. **E** is wrong: development is part of the full history but does not change immediate treatment.

4. E · Ask about each vaccine by age and site, look for a BCG scar, and record 'immunisation status unknown, no card; BCG scar present' with a catch-up plan

E is correct: without a card, ask by **age and site**, check the **BCG scar**, document the uncertainty and plan catch-up at discharge. A BCG scar matters in a child with suspected TB. **A** is wrong: unverified reports are often inaccurate. **C** is wrong: assuming no vaccines leads to unnecessary injections; follow catch-up guidance. **D** is wrong: immunisation affects the differential at any age (measles, pertussis, TB). **B** is wrong: the information is needed now.

5. D · A detailed feeding history with 24-hour dietary recall, breastfeeding history and feeding during illness

D is correct: in a child with poor growth, the **feeding history** (breastfeeding, timing and type of complementary foods, meal frequency, dietary diversity, 24-hour recall, feeding during illness) explains the problem and guides counselling and nutritional rehabilitation. **A** and **E** are wrong: social factors matter, but alone they do not define the feeding problem. **C** is wrong: early milestones are part of the full history but less useful for the plan. **B** is wrong: asthma history is irrelevant here.

6. B · Explain to both that it is routine to see adolescents alone for part of the visit, then with the girl alone explain confidentiality and its limits and use HEEADSSS

B is correct: adolescents disclose more when seen **alone** with **confidentiality** explained (including its limits for serious harm); **HEEADSSS** structures the psychosocial history from less to more personal topics. **A** is wrong: she is unlikely to answer honestly and it breaches her privacy. **C** is wrong: missed periods require a sexual history (pregnancy, defilement concerns). **D** is wrong: the mother may not know and it bypasses the patient. **E** is wrong: testing without consent violates her autonomy and trust.

7. E · His corrected age is about 22 months; not walking by 18 months (corrected) is a red flag, so he needs a full developmental and neurological assessment

E is correct: born 10 weeks early, his **corrected age is about 21–22 months**; **not walking by 18 months corrected** is a red flag, and his history (very preterm, NICU, significant jaundice) raises the risk of **cerebral palsy** or other disability. **A** is wrong: the corrected age is miscalculated. **C** is wrong: waiting delays therapy. **D** is wrong: correction is applied until 2 years. **B** is wrong: there is no history of seizures.

8. C · Treat the fracture, admit for safety, document the histories verbatim with a body map, inform the senior and involve the social worker/probation officer

C is correct: an **inconsistent history**, **delayed presentation** and **bruises of different ages** are red flags for **non-accidental injury**: ensure safety (admit), document carefully, escalate to the senior and child protection services (social worker, probation and social welfare officer; police as needed). **A** is wrong: the child may be returned to danger. **B** is wrong: confrontation by a student can escalate risk and is not your role. **D** is wrong: follow the institutional process with the senior (who will involve police where appropriate). **E** is wrong: bruises of different ages on the back are not typical accidental bruising.

9. B · Sit down, listen without interrupting, acknowledge her frustration and apologise for the wait, then explain the plan

B is correct: an angry parent is usually a frightened parent; **listening, acknowledging the emotion and apologising** for the delay de-escalates, after which you can explain the plan. **A** is wrong: it dismisses her concern. **C** is wrong: avoidance worsens the situation (involve a senior if needed, but respond first). **D** is wrong: it risks the child leaving without treatment. **E** is wrong: blaming colleagues is unprofessional and does not help the child.

10. A · Allow silence, acknowledge her emotion with empathy, and give her time before discussing next steps

A is correct: the **E (Emotions)** step of SPIKES: allow silence, respond with empathy, then move to **Strategy** (same-day ART, support, partner testing) when she is ready. **B** is wrong: information given during acute distress is not heard. **C** is wrong: it dismisses her feelings. **D** is wrong: judgemental and irrelevant now. **E** is wrong: the infant needs ART the same day and the mother needs support.

11. D · Risk of bleeding, sepsis, tetanus and anaemia from 'ebiino' extraction: examine the gums, check Hb, assess tetanus risk and treat infection

D is correct: **'ebiino' (false teeth) extraction** of canine tooth buds is still practised in parts of Uganda and causes **bleeding, local and systemic infection, tetanus and anaemia**; it changes your examination and treatment. Counsel the family respectfully. **A** is wrong: it is harmful. **C** is wrong: teething does not cause significant diarrhoea; the diarrhoea needs its own assessment. **B** is wrong: the baby needs medical care now. **E** is wrong: education and counselling are appropriate; this is not usually managed as a police matter.

12. E · Pale (white or clay-coloured) stools and dark urine

E is correct: **prolonged jaundice (>14 days) with pale stools and dark urine** suggests **conjugated hyperbilirubinaemia** (biliary atresia until proven otherwise): check conjugated bilirubin urgently and refer (surgery before about 60 days improves outcome). **A**, **C** and **B** are wrong: they do not change the investigation. **D** is wrong: a social smile is expected at about 6 weeks and does not direct the jaundice work-up.

13. C · Explore barriers to adherence non-judgementally, and work with the caregiver and counsellor towards full disclosure of his HIV status

C is correct: at 13 he should already know his status (full disclosure is usually aimed for by about 10–12 years); lack of knowledge undermines adherence. Explore barriers kindly and plan **disclosure with the caregiver and counsellor**. **A** is wrong: disclosure should be planned with the caregiver and counselling support, not abrupt. **B** is wrong: blame worsens adherence. **D** is wrong: adolescents need to own their treatment. **E** is wrong: switching requires intensive adherence counselling and repeat viral load per guidelines.

14. A · The weight and nutritional status (Z-scores/MUAC), immunisation status, HIV exposure and feeding

A is correct: the opening line of a paediatric presentation includes **age, sex, weight and nutritional status, immunisation status, HIV exposure and feeding**, because these shape the differential and the plan. **B** is wrong: tribe and religion are context, not first-line information. **C**, **D** and **E** are wrong: they do not help clinical reasoning.

15. A · What fuel does the family use for cooking, and where is it used?

A is correct: **indoor smoke from charcoal or firewood** (and tobacco smoke) is a major trigger for asthma and pneumonia; reducing exposure is part of management. **B** is wrong: exercise history matters for exercise-induced symptoms but is less important here. **C**, **D** and **E** are wrong: they do not affect asthma management.

16. C · Ask her to show you how she will prepare a dose and tell you when and for how many days she will give it

C is correct: **teach-back** (demonstration and explanation in her own words) is the most reliable check of understanding. **A** is wrong: most people say 'yes' even when unsure. **B** is wrong: she may not read English, and a leaflet does not check understanding. **D** is wrong: you remain responsible for clear communication. **E** is wrong: a signature does not show understanding.

SELF-TEST

Clinical cases

Work through each step before reading the model answer.

Case 1: The thin toddler with a coughing grandmother

A 2-year-and-3-month-old boy is brought to the Mengo paediatric clinic by his mother with cough for 4 weeks, fever for 2 weeks and weight loss. Weight 9.8 kg, height 82 cm, MUAC 12.3 cm. He is alert with no danger signs.

1. How do you begin, and who is your informant?

> Greet the mother and child, introduce yourself, confirm she is the **mother and main carer** (reliable), check triage (no emergency signs), settle the child on her lap and **count the RR** before disturbing him. Begin with an open question: 'Tell me about his cough.'

2. Which HPI details and contacts will you ask about?

> Cough character and duration, fever pattern, night sweats, weight loss (clothes loose?), appetite, breathing difficulty, wheeze, choking episode (foreign body), diarrhoea; **danger signs**; **treatment already given** (antibiotics, antimalarials, herbs); **contacts**: anyone coughing at home, anyone on TB treatment. The mother reveals the **grandmother, who shares his room, has coughed for 3 months**.

3. What paediatric extras do you take, and why each?

> **Perinatal** (term, SVD, 3.2 kg, cried at birth), **feeding** (breastfed 14 months; now 2 meals/day of porridge and matooke, rarely beans or animal foods: poor diversity), **immunisation** (card: fully immunised, BCG scar present), **development** (walked 13 months, phrases now), **HIV** (mother tested negative 2 months ago), **family** (siblings well), **social** (one room for 5 people, charcoal stove indoors).

4. Summarise the history in one line and give your differentials.

> 'A 2-year-old fully immunised, HIV-unexposed boy with moderate wasting and poor dietary diversity, presenting with 4 weeks of cough, 2 weeks of fever and weight loss, with a likely untreated TB contact in his room.' Differentials: **pulmonary TB**, persistent bacterial pneumonia, HIV-related disease (re-test if any doubt), foreign body, asthma less likely.

5. What do you tell the mother at the end, and how do you check understanding?

> Explain in plain language that the cough may be **TB caught from the grandmother**, that tests (CXR, Xpert on gastric aspirate or stool, HIV test) are needed, that TB is curable, and that the grandmother needs testing too; explore **ICE** (she fears he will die like a cousin); feeding advice; ask her to **teach back** the plan and when to return.

Take-away: A careful contact, feeding and social history turned a 'chronic cough' into probable TB with malnutrition and a household source.

Case 2: A developmental concern in an ex-preterm

A 20-month-old girl is brought because 'she is not walking like her cousins'. She was born at 31 weeks and spent 5 weeks in the neonatal unit at a regional referral hospital. Her mother says she sits well, crawls on her tummy and says 'mama'.

1. What is her corrected age and is this a red flag?

> Born 9 weeks early: **corrected age ≈ 18 months**. **Not walking at 18 months corrected** is a red flag; combined with prematurity it needs full assessment.

2. Which perinatal and neonatal details do you take?

> Why born early (pre-eclampsia, PROM, infection), antenatal steroids, birth weight, **resuscitation at birth**, oxygen/CPAP, **convulsions**, **jaundice needing phototherapy or exchange**, sepsis/meningitis, feeding difficulties, length of stay, follow-up clinic attendance.

3. How do you take the developmental history across the four domains?

> Gross motor (head control, sitting, crawling style, pulling to stand), fine motor (**hand preference?** reaching, pincer grasp), speech/hearing (babble, words, response to name), social (smile, waving, play). Ask about **regression**. Mother reports she uses only her **right hand** since 9 months.

4. What does the early hand preference suggest and what else do you ask?

> Early (before 12–18 months) hand preference suggests **weakness of the left side**: **hemiplegic cerebral palsy** (e.g. from periventricular injury or neonatal stroke). Ask about stiffness, feeding/swallowing, seizures, vision and hearing, drooling, constipation.

5. How do you communicate your concern to the mother?

> Ask what she has noticed and what she fears (ICE). Explain honestly that her development is slower than expected and that some babies born early have **brain injury affecting movement**, that the doctors will examine her and arrange **physiotherapy and further assessment** (hearing, vision), and that early therapy helps. Avoid labels until assessed; give time for questions; arrange follow-up.

Take-away: Correct for prematurity, take milestones in all four domains, and treat early hand preference and late walking as red flags.

Case 3: The adolescent with abdominal pain: seeing her alone

A 15-year-old girl is brought by her mother to the adolescent clinic with lower abdominal pain for 2 weeks and vaginal discharge. Her mother answers all the questions. The girl looks at the floor.

1. How do you set up the consultation?

> Introduce yourself to **both**, take the main history with the mother present, then explain: 'With all young people we spend part of the visit alone; it's routine.' Ask the mother to wait outside.

2. What do you say about confidentiality?

> 'What you tell me is private. The only time I would share it is if I thought you or someone else was at risk of serious harm, and I would talk with you first.' Check she understands.

3. Which HEEADSSS questions do you ask, and in what order?

> Start with **Home** and **Education** (school, friends), then **Eating**, **Activities**, **Drugs** (alcohol, bhang), then **Sexuality** (relationship, sex, consent, condoms/contraception, STI symptoms, last period), **Suicide/mood**, **Safety** (anyone hurting her, forced sex).

4. She discloses that a 32-year-old neighbour has been having sex with her for 3 months. What are your responsibilities?

> This is **defilement** (sexual activity with a person under 18) and abuse: thank her for telling you, explain that you must involve others to keep her safe, **inform the senior**, involve the **social worker / probation and social welfare officer** and police per protocol; **examine with a chaperone**, test for **pregnancy, HIV, syphilis and other STIs**, treat STIs (PID likely), offer HIV PEP/emergency contraception if the last contact was within 72 h/5 days; counselling; safety planning before discharge.

5. How do you involve her mother?

> Discuss with the girl first what will be shared and how; with her agreement and a counsellor present, explain the situation to the mother sensitively, focusing on support and safety rather than blame.

Take-away: Adolescents need time alone with clear confidentiality, a HEEADSSS history and a safeguarding response when abuse is disclosed.

References

1. Uganda Clinical Guidelines 2023 (IMCI sick child, routine childhood vaccination, nutrition). *Ministry of Health Uganda*, 2023. https://library.health.go.ug/sites/default/files/resources/Uganda%20Clinical%20Guidelines%202023.pdf
2. Pocket book of hospital care for children, 2nd edition. *World Health Organization*, 2013. https://iris.who.int/handle/10665/81170
3. Updated guideline: paediatric emergency triage, assessment and treatment (ETAT). *World Health Organization*, 2016. https://iris.who.int/handle/10665/204463
4. Engaging with and assessing the adolescent patient (HEEADSSS). *The Royal Children's Hospital Melbourne clinical practice guidelines*. https://www.rch.org.au/clinicalguide/guideline_index/Engaging_with_and_assessing_the_adolescent_patient/
5. WHO guideline for complementary feeding of infants and young children 6–23 months of age. *World Health Organization*, 2023.
6. Consolidated guidelines for the prevention and treatment of HIV and AIDS in Uganda. *Ministry of Health Uganda*, 2022.
7. Illustrated Textbook of Paediatrics (Lissauer & Carroll), history taking and development chapters. *Elsevier*, 2021.

A study aid, not a protocol. Check doses against the Uganda Clinical Guidelines 2023, your unit's protocol and a senior before you treat a patient.